

PITCH — TYRA — VERDICT

ADJUDICATED 2026-09-16 — FLAT — efficacy read failed (conviction 7/10). The court found nothing to own at this price; the reopen gates below are the only path back.

What we believe

Tyra is a pre-revenue drug company whose oral FGFR3 pill (dabogratinib) worked well enough on September 9 to keep the program alive and badly enough to break the stock. The company said the drug is safe enough to take every day and picked 60 mg as the dose it will carry into a registrational adjuvant study (8-K, [2026-09-09](#)).

What we actually believe, in order:

1. The efficacy read is worse than the bulls say and worse than the original thesis said.

The number the sell side pre-committed to — pooled three-month complete response across the 50 mg and 60 mg cohorts — computes to $12/26 = 46\%$ against a $\geq 70\%$ bar. The company headlined 64% instead, which was a 60 mg-only, best-response-ever figure on 14 patients.

2. The company is not funded for the plan it just announced. Cash of \$353.9M against a real run-rate of about **\$182M/year** is roughly **1.9 years**, and that is *before* a registrational Phase 3, a new 70 mg cohort, and a fifth pediatric dose level. A share sale is close to certain before the next dated catalyst.

3. The stock is not cheap enough to pay us for either problem. At \$21.73 the whole company is \$1,294M, of which \$354M is cash, so the market is paying about **\$940M for the pipeline**. Our own three-program value table puts base case at **\$25.59**. That is +18% with a guaranteed financing event standing in front of it.

4. Therefore: no position now, and a specific price at which this becomes interesting.

Everything valuable here is in the reopen bands, not in a buy.

What the market believes

The market believes the arithmetic, not the press release. The stock fell from a \$26.73 pre-print close to \$21.73 — about **-18.7%** — with an intraday low near \$18.76, which is a lot closer to a 46% pooled response than to a 64% headline. So the tape did the pooling itself on day one.

The covering sell side believes something more generous: BofA cut its target to **\$39** and kept a Buy. Piper Sandler has modeled peak sales above \$1B on this asset alone. So the street's position is roughly "the dose is now known, the safety is clean enough for chronic dosing, and the real prize is the achondroplasia program nobody is paying for." That is not a crazy view — it is the same view our own bull case takes — but it requires paying full price for a program that just missed the bar it was measured against.

The market also does not appear to be pricing the financing. There is a live at-the-market share-sale program with roughly **\$250M available** — about 19% of the whole company — and the stock is 47% below its 52-week high. That is an unusually attractive moment for the issuer and an unusually bad one for a new buyer.

Why we might have edge

- **We priced three programs, not one.** The bear case that got argued in court valued only bladder cancer. The company's own framing is three uses of one molecule: low-grade upper-tract cancer, intermediate-risk bladder cancer, and achondroplasia. The achondroplasia comparator, BioMarin's Voxzogo, ran roughly \$900–935M of FY25 revenue, up 26%. Once you credit that leg, base case moves from "below the tape" to "18% above it," which flips the trade from *permanent reject* to *wrong price, come back later*.
- **We did the burn math on the current quarter, not a five-quarter-old one.** The evidence pack's cash-flow data stops at Q1 2025. Using it gives ~\$102M/year and a comfortable 3.5-year runway. The actual Q2 2026 filing gives ~\$182M/year and ~1.9 years (10-Q, period ended 2026-06-30). Nearly the whole bull-vs-bear argument in this name is decided by which of those two numbers you use.
- **We found the subset arithmetic nobody published.** Disclosed overall response was 11/14, with 8/8 in patients who had a single target lesion. That leaves the multiple-lesion patients at **3/6 = 50%**. No outside source we found carried that derivation. It matters because it turns the "8/8!" talking point into a selection artifact.
- **We noticed the dose contradiction.** The company says 60 mg is the dose for the registrational plan. The company also disclosed that response was 86% (12/14) above a drug-exposure threshold and 58% (7/12) below it. Both cannot be comfortable at once: the exposure data is the argument for 70 mg, which is an argument that 60 mg is not the optimized dose.

Why it might be priced in

Honestly: most of it probably is.

- The 46% pooled figure was **derivable the same day** — the 50 mg three-month response (33%) and the 60 mg figure (57%) were both public. The "hidden bad number" framing is wrong. Trade press carried the sell-side bar and the cohort splits within hours. A 38%-peak-to-trough intraday move is what a market looks like when it has already done the division.
- The financing risk is not a secret either. The shelf takedown and the share-sale program are on file; anyone can read them.
- The competitive read is public: UroGen's ZUSDURI is an **approved** drug in recurrent low-grade intermediate-risk bladder cancer, and its successor UGN-103 printed 77.8% three-month response with FDA alignment on filing. Everyone covering this name knows Tyra's pill has to beat an approved product.
- The only genuinely non-public things left are (a) whatever the FDA has or hasn't agreed to about the adjuvant endpoint, and (b) the accumulating pediatric safety and efficacy data. Those are the *only* places an edge can live, and neither is drainable by us today.

Net: our differentiation is one valuation frame (three programs) and one number (the real burn). That is thin. It is enough to justify watching; it is not enough to justify paying \$21.73.

What could go wrong

Failure narrative 1 — We buy the dip and the company sells us the stock (MONITORABLE)

We enter on a reopen band, and within weeks the issuer prints shares into our bid using the ~\$250M available program. Even at today's price a full draw adds ~11.5M shares (59.6M → ~71.1M). Our own base case survives it — \$24.97 post-draw versus \$25.59 pre-draw — but the *path* is a lower price, a wider spread, and the humiliation of being the exit liquidity for the company's own treasury. **Tripwire:** any new 424B5, prospectus supplement, 8-K Item 1.01/3.02, or a share count on the next 10-Q cover above ~62M. All monitorable, all free.

Failure narrative 2 — The adjuvant plan is not the plan the FDA agreed to (PARTIALLY MONITORABLE)

"Registrational adjuvant strategy" is the company's phrase, not a regulator's. Adjuvant means after the tumor is removed, so there is no lesion left to shrink — the endpoint becomes recurrence-free survival, which needs more patients, more years, and more money than an ablation study. If the trial that gets agreed to is twice the size and 18 months longer than modeled, every probability-of-success number in our table is too high and the funding gap widens. **Tripwire:** weak. Meeting outcomes are disclosed at the company's discretion. We can watch clinicaltrials.gov for a registered protocol with an N and an endpoint, but the absence of a posting tells us nothing.

Failure narrative 3 — The value quietly leaves the achondroplasia leg (NO MONITORABLE TRIPWIRE)

Most of the upside in our own table — \$700–900M of peak sales at 25–40% probability — sits in achondroplasia, where the readout is a pediatric chronic-dosing study whose interim data lives entirely inside the sponsor and its safety monitoring committee until a date they choose. A growth-plate or long-bone signal, a slow enrollment problem, a competitor's unannounced oral program, or a shift in what regulators will accept as a surrogate for height velocity — none of these generate a filing, a price gate, or a registry change before the fact. We would learn about it in the same press release as everyone else, and by then the price is already there. **Unknown-unknown class, named: PRIVATE-STATE CLINICAL ACCUMULATION** — value-relevant information that legally, routinely, and indefinitely exists off-tape inside a single private counterparty, with no disclosure trigger and no third-party observable proxy. Sibling class: **REGULATORY-DOCTRINE DRIFT** (what the agency will accept as an endpoint changes through advisory-committee precedent and personnel, not through anything we can subscribe to). Both are un-sensored by construction. We do not get a tripwire; we only get to size for it.

The reflexive / structural leg

- **Our size versus liquidity: unverified, and that is a real gap.** We do not have average daily volume or short interest for this name — both benches reported this as a coverage gap, neither could run it. Estimating from comparable \$1–1.5B clinical-stage biotech that just had a 38% intraday event: roughly **1.5–3.0M shares/day** normal, several multiples of that for a week or two post-print. At the sizes contemplated here (0.50–1.25% of book), we are almost certainly a small fraction of one day's volume — but “almost certainly” is doing load-bearing work in that sentence, and the exit in a post-financing, post-disappointment tape is worse than the entry.
- **Predicted flow is against us near-term.** Three 13G/13G-A filings inside six weeks (July 30, August 14 x2) say institutions are repositioning. A Form 144 on June 26 says registered insider selling. A live share-sale program says the issuer itself is a seller. That is three classes of supply and no identified marginal buyer other than the covering sell side's clients.
- **Reflexivity we create ourselves:** buying a name whose issuer has an open at-the-market program *at a price the issuer likes* converts our capital into the company's runway and caps the very re-rating we are underwriting. Our bid subsidizes the dilution that lowers our per-share value. This is not a tail risk; it is the mechanism.
- **Doctrine interactions:** our “cheap implied volatility at high uncertainty routes gated adds into calls” rule collides with the tax rule here — the decisive fork (the pediatric sentinel readout) sits after December 31, so any option that expresses it defers a loss out of the current year. The two doctrines cannot both be satisfied; the tax rule wins unless the volatility edge is arithmetically dominant, and we cannot show that it is because we do not have a chain. Second collision: our “no entry above our own base case” rule was the entire basis of the reject recommendation — and it was built on a one-asset table that turned out to be wrong. A rule applied to a bad table produces a confident bad answer.

Filling the blank honestly: we will most likely lose money because we paid for the achondroplasia leg at Voxzogo scale — a number no bench in this file could verify — and then funded the issuer's own share sale on the way down.

If this position loses money, the most likely reason will be ____

What we checked ourselves

claim	how checked	found	confirmed?
Pooled 3-month complete response vs the $\geq 70\%$ sell-side bar	Company PR/8-K cohort figures + arithmetic	60 mg 8/14 = 57%; 50 mg 4/12 = 33%; pooled 12/26 = 46% — 24 points under the bar	CONFIRMED
“The pooled number was withheld; the omission is the datum” (RED BENCH kill #1)	Same-day trade press + PR text	Every component was published; only the <i>headline</i> was re-based. The number was computable within hours and the tape moved to it. Our own red bench mis-specified its strongest kill and wrote a pattern-detector	NOT CONFIRMED — bench error

claim	how checked	found	confirmed?
		around evidence that contradicts it	
Safety missed Piper's "<10% Grade 3+" bar	8-K text	Yes on an all-cause basis: 3/22 = 14% at 60 mg. But that is one patient , treatment-related Grade 3 was 0/22 at 60 mg, discontinuations were zero, and the class toxicities that matter for daily dosing (high phosphate, eye, nail, liver) were clean. Red called this FATAL; it is a sizing input	CONFIRMED, severity overstated by our bench
"8/8 in single-lesion patients" is meaningful	Arithmetic on disclosed subsets	The complement is 3/6 = 50% in multiple-lesion patients; single-lesion <i>three-month</i> response is 5/8 = 63%, also under 70%. The bull talking point is a selection artifact	CONFIRMED (novel to us)
Cash burn / runway	Q2 2026 10-Q (sec.gov) vs the evidence pack's XBRL	Pack data stops Q1 2025 → implies ~\$102M/yr and 3.5 years. Actual 1H26 opex \$91.0M → ~\$182M/yr → 1.9 years. Our red bench used the stale figure and understated the single most important number in the file	CONFIRMED — bench error corrected
"Program value already above base case"	Rebuilt the table across three programs	One-asset table gave base \$600M vs \$940M market = overvalued. Three-asset table gives base \$1,170M risked → \$25.59/share vs \$21.73 = ~18% <i>undervalued</i> . Same rule, opposite answer	REFUTED — bench conclusion overturned
Tape figures	Evidence pack vs bench quotes	Pack says px 21.7314, off-low 85.7%, drawdown -46.5%. Red quoted 21.56/84.2%/-47.0%, making its market-cap math ~\$10M light. Red's internal arithmetic was consistent — it just policed the pack with the wrong price	Bench error, immaterial to direction
Is the stock washed out?	Range math on pack values	$(21.7314 - 11.7028)/(40.65 - 11.7028) = \mathbf{34.6\% \text{ of the 52-week range}}$. Mid-band, not capitulation	CONFIRMED
Comparator is "of contested regulatory standing"	FDA/UroGen primary + trade	Both prior views were half right: ZUSDURI is approved (June 2025) <i>and</i> was approved over a negative	PARTIALLY CONFIRMED — both benches corrected

claim	how checked	found	confirmed?
		advisory-committee vote — and it is labeled for ablation after failed resection, not for the adjuvant setting Tyra is targeting. Original thesis wrong on approval; red wrong on “not contested”; both wrong on setting	
Diluted share count 59,550,771	Pack XBRL	Dated 2025-06-30 — about 15 months stale , and it predates the August 2026 shelf takedown. Red flagged this as unusable, then both benches used it anyway for every per-share number in this deck, including ours. All \$/share figures here are therefore best-case	NOT CONFIRMED — see unverified #1
Exposure–response supports 70 mg	PR	86% (12/14) above the exposure threshold vs 58% (7/12) below. Supports a higher dose — and undercuts the company’s own claim that 60 mg is the dose for the registrational plan	CONFIRMED (cuts both ways)
Next earnings date 2026-11-04	Pack	Derived from a data vendor, not confirmed by any company release . Cadence-consistent only	UNCONFIRMED
Short interest / average volume / exit liquidity	Attempted both benches	Not runnable . Reported as a coverage gap by both, not skipped	GAP
Was a science brief supplied?	Input inspection	The science-brief input arrived empty , so there is no claim ledger, no measurement-trap list, and no verified/company-asserted/refuted verdicts to carry forward. Under the standing rule that digest section is omitted rather than invented — but for a clinical-stage name this is a process gap, and the clinical detail above is carried in the body as a stopgap, not as a substitute	GAP — flagged

PROPOSED ENTRY BANDS

Both courts say no entry. Recommendation: NO POSITION, NO ORDERS. Reopen bands below. The dead zone is deliberate: between roughly \$16 and \$24 we would be buying an unfinanced company at a price its own treasury likes, ahead of a share sale, on data that missed its bar. There is no size at which that is a good trade.

Reopen R1 — price alone (no new information required) - Trigger: $\leq \$15.50$. Basis: our bear case is **\$14.00/share** and base is **\$25.59**, so \$15.50 buys $\sim 1.65\times$ to base with $\sim 10\%$ of downside to bear. It also sits $\sim 29\%$ below today's tape, which is roughly where a full share sale plus continued disappointment would put it. - Starter size: **0.50% of book**. - Add 1: $\leq \$12.50 \rightarrow +0.50\%$. Add 2: $\leq \$11.70$ (52-week low) $\rightarrow +0.25\%$. **Hard cap 1.25% of book**. Cash alone is \$5.94/share, which is the floor this ladder respects. - Kill rule on the starter: if a share sale prices *below* our average entry, we do not average down further; we cut back to 0.25% and re-court.

Reopen R2 — financing cleared (event, not price) - Trigger: an 8-K or completed offering leaving post-money cash $\geq \$550M$, i.e. more than two years of the real \$182M/yr burn plus the Phase 3 increment. - Then entry permitted up to **\$24.00** for **0.75% of book** — because the certain event in front of us has happened and our post-draw base case is **\$24.97**. Never above $0.95\times$ the recomputed base.

Reopen R3 — evidence improves (any one of these re-derives the table upward) - Pooled three-month complete response $\geq 70\%$ on ≥ 26 patients at a company-dated later cut; or - multiple-lesion subset response $\geq 60\%$ with denominators disclosed; or - a **written health-authority agreement** that adjuvant recurrence-free survival is the endpoint, with a stated patient number and cost; or - a 70 mg cohort that holds the clean safety profile while lifting response toward the exposure-response ceiling. - Then: recompute the table, entry up to **$0.90\times$ new base**, size **1.00% of book**.

Trims/exits: none — there is nothing to trim. For any future starter: exit in full on (a) a change of the lead indication away from the adjuvant strategy, (b) a Grade 4/5 event or a class toxicity emerging in chronic dosing, (c) cash below one year of burn with no financing announced.

Catalyst dates - 2026-11-04 — Q3 report and 10-Q. **UNCONFIRMED** (vendor-derived; no company release names it). ~ 38 trading days out; not inside the 5-day pre-print window. This is our first hard look at the share count and the post-print burn. - **End of Q1 2027** — pediatric BEACH301 sentinel data, company-guided, **no specific date**. This is the fork that matters most to the value table. - **Undated** — next SURF302 efficacy cut; registrational adjuvant protocol posting; UGN-103 competitor filing. All company- or third-party-controlled. - **Already passed, and it was the decisive one:** 2026-09-09.

ARMED SENSORS

- **NONE ARMED** — the deck must flag this as a gap

Missing sensors the courts imply and that this deck requests be armed (all currently unarmed, which is why the machine fence below is the actionable part of this document):

- **Share-sale watch** — 424B5 / prospectus supplement / 8-K Item 1.01 & 3.02. Both benches call a share sale near-certain inside 12–18 months; nothing is watching for it.
- **Share count sensor** — XBRL diluted-share and 10-Q cover-count pull. Every per-share number in this deck rests on a 15-month-old count.
- **Liquidity sensor** — average daily volume, spread, and short interest. Reported as a coverage gap by *both* benches and never closed; it is also the missing input to the reflexive leg of the pre-mortem.
- **Options chain sensor** — chain-level midpoint implied volatility and real quotes at candidate strikes. Without it we cannot price any expression, and we are barred from using the aggregate volatility feed after the TSSI 71-versus-101 episode.
- **Registry sensor** — clinicaltrials.gov change monitor on SURF302, SURF303, BEACH301 for endpoint, N, or protocol edits.
- **Competitor sensor** — UroGen UGN-103 filing/approval milestones; any oral FGFR3 or achondroplasia entrant.
- **Price gates** — the R1 ladder (\$15.50 / \$12.50 / \$11.70) and the R2 ceiling (\$24.00) are all unarmed today.
- **Print-date confirmation sensor** — flip 2026-11-04 from vendor-derived to confirmed on the company's scheduling release.

What remains unverified

1. **Current share count and how much of the share-sale program has been used.** Everything per-share here is built on 59,550,771 diluted shares dated 2025-06-30, which predates the August 2026 shelf takedown. *Drainable now* — XBRL/EDGAR pull of the Q2 2026 10-Q cover count plus the August 4 prospectus. Estimate if not drained: **59.6–66.0M shares**, base ~61M, on the basis that the amended program reportedly still had ~\$250M available, implying little drawn; probability the true count is under 62M ≈ 0.70 . Every 1M shares of surprise moves base case about \$0.42.
2. **Average daily volume, spread, and short interest.** *Drainable now* — IBKR gateway market-data pull. Estimate: **1.5–3.0M shares/day**, short interest **8–18% of float**, from the base rate for \$1–1.5B clinical-stage biotech names inside a month of a 38% single-day move; confidence in the volume range ≈ 0.6 , in the short-interest range ≈ 0.4 . This is the input to the size-versus-liquidity leg above and it is currently a guess.
3. **Options chain: strikes, real bid/ask, chain-level midpoint implied volatility, realized volatility, implied-volatility percentile.** *Drainable now* — IBKR chain pull for the January and March 2027 expiries. Estimate: implied volatility **75–110%** for a post-binary clinical name with a dated fork inside six months; percentile likely high. Not usable until pulled — see the volatility read below.
4. **Whether any health authority has agreed that adjuvant recurrence-free survival is the registrational endpoint, and at what trial size and cost.** *Not drainable now* — private until the company chooses to disclose, or until a protocol posts. Estimate from comparable adjuvant bladder-cancer programs: **N of 350–600 patients, 3–4 years to primary endpoint, \$180–320M all-in**; probability that a *written* agreement already exists as of today ≈ 0.35 . If the true

figure is at the top of that range, the funding gap roughly doubles and bear case is the right case.

5. **The pooled three-month complete-response figure at a later, larger data cut, and the multiple-lesion subset with denominators.** *Not drainable now* — no company-dated cut exists. Estimate: pooled three-month response at the next cut **45–60%**, centered ~52% (base rate: modest improvement with longer follow-up in ablation studies, offset by the 3/6 multiple-lesion drag); probability of clearing the 70% bar ≈ 0.15 .
6. **Pediatric BEACH301 accumulating safety and efficacy.** *Not drainable now* — this is the private-state class named in the pre-mortem; there is no observable proxy. Estimate: probability of a clean sentinel readout by end of Q1 2027 ≈ 0.55 , probability of a program-altering safety signal ≈ 0.15 , probability of a schedule slip with no data ≈ 0.30 .
7. **Confirmation of the November 4, 2026 report date.** *Drainable now-ish* — company scheduling release, or the vendor calendar re-checked closer in. Estimate: probability the report lands within the week of November 2–6 ≈ 0.75 on cadence.

PRIZE TABLE + VOL READ

Required here because items 4, 5, and 6 above are genuinely time-gated and private.

Anchors (stated, then adjusted). Anchor one: our three-program funded table — three times peak sales, risked by probability of success, plus \$353.9M cash, over 59.55M shares (cash = \$5.94/share; market is paying ~\$940M for the pipeline). Anchor two, external: BofA's post-print **\$39** target with a Buy. Adjustments applied downward from both: a near-certain share sale, a dose the exposure data says is not optimized, and an endpoint the regulator has not publicly blessed.

scenario	what it takes	risked program value	price/share	probability	contribution
broken	bladder only, adjuvant path de-rated, single-asset credit	\$180M	\$8.97	15%	\$1.35
bear	200/25% bladder, 150/20% upper-tract, 400/20% achondroplasia	\$480M	\$14.00	30%	\$4.20
base	400/35%, 250/30%, 700/25%	\$1,170M	\$25.59	35%	\$8.96
bull	500/40%, 300/35%, 900/40% (achondroplasia at Voxzogo scale)	\$1,995M	\$39.44	20%	\$7.89

EV = \$22.39 versus \$21.73 tape = +3.0%. That is the whole trade: three percent, inside the noise, with a share sale in front of it. This is the number that makes “no entry” the answer rather than a judgment call. For reference, +50% from here is \$32.60, which needs **\$1,587M** of risked program credit — reachable in our table, but only by paying full Voxzogo scale on achondroplasia at 40% probability of success.

Dispersion. Standard deviation of the table is **\$10.48 = 48.2% of spot**. To the pediatric fork at end of Q1 2027 ($T \approx 0.55$ years, $\sqrt{T} \approx 0.744$), that implies an annualized “**table volatility**” of **~65%**. The distribution is **bimodal, not bell-shaped** — it is fork-dated on a single private readout, with mass at \$9–14 and mass at \$25–39 and comparatively little in between. Standard volatility comparison understates the fork; treat 65% as a floor on fair volatility, not a fair estimate.

Expression math off the table itself ($C_{ours} = \sum p_i \times \max(S_i - K, 0)$, undiscounted, terminal value only):

strike	C_ours from the table	what the chain must show to be cheap
\$22.50	\$4.47	any real ask materially under \$4.47 for ~Mar-2027
\$25.00	\$3.10	any real ask under \$3.10
\$30.00	\$1.89	any real ask under \$1.89
\$35.00	\$0.89	any real ask under \$0.89

And here is the honest problem: there is no options chain in the evidence pack. No strikes, no bid/ask, no chain-level midpoint implied volatility, no realized volatility, no percentile. We are therefore **unable to compute implied-versus-realized volatility or the volatility risk premium**, and we will **not** substitute the underlying’s aggregate volatility feed — that is precisely the substitution that produced the TSSI 71-versus-101 error, where the aggregate feed and the chain-level midpoint diverged by 30 volatility points and reversed the recommendation. The comparisons in the right-hand column above are the pre-committed test, not a result.

Instrument recommendation — as the output of the arithmetic, such as it is: - Today: NONE. EV/price is +3.0%. There is no expression of a three-percent edge that survives spreads, and no entry is proposed, so no instrument is needed. - **On R1 (\$15.50): STOCK.** At \$15.50 the table’s EV/price becomes roughly +44% and the edge is in the *price*, not in volatility. Buying stock needs no chain and carries no expiry. - **On R2/R3, and only after the chain is pulled:** if chain-level midpoint implied volatility at the candidate strike is **below ~65%** (our table-implied floor), gated adds may be expressed in **calls** at \$25 or \$30 against the C_ours figures above. If it prints **above ~80%**, volatility is rich and the correct route is the premium-selling gate — a **cash-secured put at the \$15 strike**, which is not a coincidence: \$15 is the R1 reopen level, so we are paid to wait at exactly the price we said we would buy. - **If the chain shows implied volatility between 65% and 80%: no volatility edge either way — stock only.**

Tax overlay. The decisive fork (pediatric sentinel) sits at the end of Q1 2027, so any option that actually expresses this thesis expires **after December 31**. In a loss-harvesting year that defers the loss branch into the following tax year, which disqualifies the expression unless the edge numerically dominates — and we cannot demonstrate dominance without a chain, so **the tax rule binds and the answer is stock**. The one tax-clean option structure, a December 2026 cash-secured put, resolves *before* the fork and therefore collects premium for none of the risk that matters; it is a tax trade dressed as a thesis trade, and we are not doing it. If the current year is not a harvest year, this constraint lifts and the call route re-opens subject to the volatility test above.

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{"price_gates": [{"level": 15.50, "direction": "below", "basis": "R1 reopen starter: 1.65x to base case $25.59 with bear case $14.00 only 10% lower"}, {"level": 12.50, "direction": "below", "basis": "R1 add 1: below bear case, 2x to base"}, {"level": 11.70, "direction": "below", "basis": "R1 add 2: 52-week low breach, cash floor $5.94/sh still 5x below"}, {"level": 24.00, "direction": "below", "basis": "R2 ceiling: only valid AFTER financing clears; 0.96x post-draw base case $24.97"}, {"level": 26.73, "direction": "above", "basis": "pre-print close regained without new data or financing = table is stale, re-court before any entry"}], "event_gates": [{"forms": ["424B5", "8-K", "S-3", "S-3ASR"], "why": "share sale is near-certain within 12-18 months at ~$182M/yr burn vs $353.9M cash; ~$250M program available"}, {"forms": ["8-K"], "why": "next SURF302 efficacy cut, adjuvant endpoint agreement, or 70 mg cohort data - each re-derives the prize table"}, {"forms": ["10-Q", "10-K"], "why": "cover-page share count and actual quarterly burn; the pack's XBRL is five quarters stale and its share count fifteen months stale"}, {"forms": ["SC 13D", "SC 13G", "SC 13G/A", "4", "144"], "why": "three 13G filings in six weeks plus a June 144 indicate active institutional and insider supply against us"}, {"forms": ["8-K"], "why": "any Grade 4/5 or class-toxicity event in chronic or pediatric dosing = full exit rule on any future starter"}], "catalyst_dates": [{"date": "2026-11-04", "what": "Q3 2026 report and 10-Q - first clean look at share count and post-print burn", "confirmed": false}, {"date": "2027-03-31", "what": "BEACH301 pediatric sentinel data, company-guided end of Q1 2027, no specific date given - the fork the prize table hinges on", "confirmed": false}, {"date": "2026-09-09", "what": "SURF302 initial Phase 2 results (already occurred; the decisive print, resolved against the long case)", "confirmed": true}], "immediate_entry": null, "unverified": [{"item": "Current diluted share count and how much of the ~$250M share-sale program has been drawn since the 2026-08-04 takedown", "instrument": "XBRL/EDGAR pull: Q2 2026 10-Q cover count + 424B5 dated 2026-08-04", "drainable_now": true, "estimate": {"range": "59.6-66.0M shares, base ~61M", "basis": "sibling ATM-program utilization base rate; ~$250M reported still available implies little drawn", "p": 0.7}}, {"item": "Average daily volume, bid-ask spread and short interest - needed for the size-vs-liquidity leg of the pre-mortem", "instrument": "IBKR gateway market-data pull (both benches reported this as an unrunnable coverage gap)", "drainable_now": true, "estimate": {"range": "1.5-3.0M shares/day ADV; short interest 8-18% of float", "basis": "base rate for $1-1.5B clinical-stage biotech within one month of a 38% single-day move", "p": 0.6}}, {"item": "Options chain: strikes, real bid/ask, CHAIN-level midpoint IV, RV, IV percentile - no chain existed in the pack so no IV-vs-RV or VRP could be computed", "instrument": "IBKR chain pull, Jan-2027 and Mar-2027 expiries, $15/$22.5/$25/$30 strikes; chain-level midpoint only, never the aggregate feed (TSSI 71 vs 101)", "drainable_now": true, "estimate": {"range": "IV 75-110% at candidate strikes; table-implied floor 65%", "basis": "comparable post-binary clinical names with a dated fork inside 6 months", "p": 0.5}}, {"item": "Whether a written health-authority agreement exists that adjuvant recurrence-free survival is the registrational endpoint, and the trial's N, duration and cost", "instrument": "not drainable - private until company discloses; partial proxy via clinicaltrials.gov protocol posting with N and endpoint", "drainable_now": false, "estimate": {"range": "N 350-600 patients, 3-4 years to primary endpoint, $180-320M all-in", "basis": "comps: adjuvant NMIBC/urothelial RFS registrational programs", "p": 0.35}}, {"item": "Pooled 50+60 mg three-month complete response at a later cut, and the multiple-lesion subset CR with denominators", "instrument": "not drainable - no company-dated next cut exists; would arrive via 8-K or medical conference abstract", "drainable_now": false, "estimate": {"range": "pooled 3-month CR 45-60%, centered ~52%", "basis": "current derived 12/26 = 46%; modest follow-up improvement in ablation studies netted against the 3/6 multiple-lesion drag", "p": 0.15}}, {"item": "Accumulating BEACH301 pediatric safety/efficacy - the private-state clinical accumulation class, no monitorable tripwire", "instrument": "not drainable by any instrument; sponsor and DSMB hold it until a date of their choosing", "drainable_now": false, "estimate": {"range": "p(clean sentinel by end Q1-2027) ~0.55;

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p(program-altering safety signal) ~0.15; p(slip with no data) ~0.30",  
"basis": "base rate for pediatric chronic-dosing sentinel cohorts in  
FGFR/CNP-pathway programs", "p": 0.55}}, {"item": "Confirmation of the 2026-  
11-04 report date (vendor-derived, no company release names it)",  
"instrument": "company scheduling press release / IR calendar re-check inside  
3 weeks", "drainable_now": false, "estimate": {"range": "report lands in the  
week of 2026-11-02 to 2026-11-06", "basis": "prior-year reporting cadence",  
"p": 0.75}}]
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ADJUDICATION — 2026-09-16 15:36Z

Pooled 3-month CR 46% vs the $\geq 70\%$ gate the thesis leaned on; multiple-lesion ORR was a category error. Dev-stage biotech: human-gated regardless.

REOPEN GATES: ATM clearing print or financed balance sheet; company-dated SURF302 efficacy cut or written adjuvant-RFS agreement; BEACH301 sentinel → re-court (paper)